

August 13, 2026

South Carolina Medicaid (Healthy Connections / SCDHHS)
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Regena Moore (MRN MUSC8962062), Claim MUSC-B4EA-2

Patient	Regena Moore (DOB 1964-12-08)
MRN	MUSC8962062
Member ID / Group	SCD742523430 / GRP-85832
Claim number	MUSC-B4EA-2
Date of service	2026-06-28
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$428.70
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2026-09-04

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health University Medical Center is submitting this formal first-level appeal on behalf of patient Regena Moore (MRN: MUSC8962062, Member ID: SCD742523430) regarding the denial of Claim MUSC-B4EA-2 for services rendered on June 28, 2026, by Dr. Marcus T. Bledsoe, DO (NPI 1770615243). South Carolina Medicaid (Healthy Connections / SCDHHS) denied the claim in full — billed charges of \$428.70 — citing CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") and RARC N115 ("This decision was based on a Local Coverage Determination (LCD)"), with the accompanying remark that "Documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." MUSC Health respectfully disagrees with this determination and requests full reversal and payment at the allowed amount of \$232.75.

CLINICAL BACKGROUND

Ms. Moore is a 61-year-old established female patient with active chronic conditions including anemia (D64.9, managed with ongoing ferrous sulfate therapy since 2001) and prediabetes (E11.9, onset 1998), both of which require periodic monitoring and clinical management. On June 28, 2026, she presented to MUSC Health University Medical Center's on-campus outpatient facility for an evaluation and management visit with Dr. Bledsoe, coded as CPT 99214 — an office or outpatient visit for an established patient involving moderate medical decision-making. The primary diagnosis recorded for this encounter was J06.9, acute upper respiratory infection, unspecified. Ms. Moore's longitudinal history reflects a pattern of recurrent acute respiratory illnesses, including prior episodes of acute bronchitis, viral pharyngitis, and viral sinusitis, as well as her active comorbidities, all of which are relevant to the clinical complexity of any acute presentation.

BASIS FOR APPEAL

The denial rests on the assertion that the documentation does not establish medical necessity for the service as billed. MUSC Health contends that this determination is inconsistent with both the clinical facts of the encounter and the applicable standards governing evaluation and management services under South Carolina Medicaid.

CPT 99214 is appropriately assigned to an established patient visit involving moderate complexity, which encompasses the assessment of an acute illness with consideration of the patient's underlying chronic conditions. Ms. Moore's active diagnoses of anemia and prediabetes represent comorbid factors that a treating physician must reasonably consider when evaluating an acute upper respiratory infection, as these conditions can influence symptom presentation, treatment selection, and patient risk. The level of service billed reflects this clinical context and is consistent with generally accepted evaluation and management coding guidelines.

Furthermore, J06.9 is a recognized, billable ICD-10-CM diagnosis code for acute upper respiratory infection, unspecified, and its use does not in itself preclude a finding of medical necessity. An acute symptomatic illness in a patient with multiple active chronic conditions constitutes a medically necessary reason for an outpatient encounter. The payer has not identified a specific documentation deficiency beyond a general assertion, and MUSC Health maintains that the medical record supports the level and nature of service provided. Should the payer require additional clinical documentation from the treating provider, MUSC Health is prepared to submit supplemental records upon request.

REQUESTED ACTION

MUSC Health respectfully requests that South Carolina Medicaid (Healthy Connections / SCDHHS) reverse the denial of Claim MUSC-B4EA-2 and reprocess the claim for payment at the applicable allowed amount of \$232.75. This appeal is submitted within the 30-day appeal window, prior to the September 4, 2026 deadline. If the payer requires additional clinical documentation or a peer-to-peer review to support this determination, please contact MUSC Health's Revenue Cycle Appeals department at your earliest convenience so that we may facilitate a timely resolution.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

Senior Appeals Specialist, Revenue Cycle Management
MUSC Health — Revenue Cycle / Patient Financial Services
(843) 792-2300 | muschealth.org

Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record